

ST. JUDE MEDICAL CENTER
Department of Neurology & Memory Disorders
1200 Healthcare Parkway, Building 8, Suite 400
Confidential Medical Record

PATIENT DEMOGRAPHICS & VISIT DETAILS

Patient Name: Miller, Robert L.		
DOB: 04/12/1948 (Age: 78)	Sex: Male	MRN: 9482-0193
Date of Examination: 08/20/2026	Attending Physician: Dr. Sarah Jenkins, MD (Neurology)	
Attending Physician: Dr. Sarah Jenkins, MD (Neurology)	Referring Physician: Dr. Alan Vance, MD (Primary Care)	

REASON FOR EVALUATION

Comprehensive neurological and cognitive evaluation for progressive memory loss, functional decline, and behavioral changes over the past 24 months.

Accompanying family member: Susan Miller (Daughter/Primary Caregiver).

CLINICAL HISTORY (History of Present Illness)

The patient is a 78-year-old right-handed male who presents with a 2-year history of gradually progressive cognitive decline. Per caregiver report, early signs included repetitive questioning and misplacing familiar household items. Over the past 8 months, the patient has developed significant executive dysfunction, including difficulty managing personal finances and confusion while driving in familiar areas.

- **Functional Assessment:** Dependent on caregiver for instrumental activities of daily living (IADLs), including medication management, driving, and meal preparation. Independent with basic ADLs (bathing, dressing, grooming), though requiring occasional verbal prompting.
- **Behavioral Symptoms:** Mild apathy, intermittent nighttime agitation (sundowning), and social withdrawal. No reports of visual hallucinations, delusions, or motor tremors.

PAST MEDICAL & NEUROLOGICAL HISTORY

- Essential Hypertension (controlled)
- Hyperlipidemia
- No history of stroke, head trauma, or major depressive disorder

MEDICATIONS

- Amlodipine 5 mg PO daily
- Atorvastatin 20 mg PO daily
- Multivitamin daily

NEUROLOGICAL & PHYSICAL EXAMINATION

General: Alert, pleasant, well-groomed male in no acute distress.

Cranial Nerve (I-XII): Intact. Visual fields full; extraocular movements intact without nystagmus.

Motor: Normal tone and bulk throughout. Strength 5/5 in all four extremities. No tremors or involuntary movements noted.

Reflexes: 2+ symmetrical throughout. Babinski reflex bilaterally flexor (normal).

Gait & Balance: Normal narrow-based gait. Casual walk and tandem gait unremarkable. Negative Romberg test.

STANDARDIZED COGNITIVE TESTING

- **Mini-Mental State Examination (MMSE):** Score 19 / 30 (Moderate Impairment)
- Orientation (Time/Place): 5/10 (Disoriented to date, day of the week, and floor level)
- Registration: 3/3
- Attention & Calculation (Serial 7s): 2/5
- Recall: 0/3 (Unable to recall 3 words after 5-minute delay)
- Language & Visual Construction: 9/9 (Clock Drawing Test showed significant spatial distortion and incorrect hand placement)

DIAGNOSTIC WORKUP

- **Laboratory Evaluation (Completed 08/10/2026):**
 - CBC, CMP, TSH, Vitamin B12, Folate, Syphilis Serology (RPR). All within normal limits. Reversible toxic/metabolic causes of cognitive decline ruled out.
- **Brain MRI (Without Contrast - Completed 08/12/2026):**
 - Findings: Symmetrical moderate-to-severe hippocampal and medial temporal lobe atrophy (MTA Score: Grade 3). Generalized mild cortical thinning. Age-appropriate white matter ischemic changes (Fazekas Grade 1). No acute infarction, intracranial hemorrhage, or space-occupying lesion.

DIAGNOSTIC IMPRESSION

- Major Neurocognitive Disorder due to probable Alzheimer's Disease (ICD-10: G30.9 / F02.80), moderate stage.
- Clinical picture characterized by insidious onset, prominent episodic memory loss, executive dysfunction, and characteristic medial temporal lobe atrophy on MRI, in the absence of metabolic or structural alternatives.

PLAN & RECOMMENDATIONS

- **Pharmacotherapy:**
 - Initiate Donepezil (Aricept) 5 mg PO at bedtime for 4 weeks; if well-tolerated without gastrointestinal side effects or bradycardia, increase up to 10 mg PO daily.
 - Plan to discuss adding Memantine (Namenda) at the 3-month follow-up visit.
- **Safety & Care Planning:**
 - Driving: Strongly advised against operating a motor vehicle due to executive impairment. Formal driving evaluation referral placed.
 - Home Safety: Recommend occupational therapy home safety evaluation (fall risk prevention, stove safety knobs).
 - Legal/Financial: Advise patient and family to establish Medical Power of Attorney (POA) and update advanced directives while the patient retains decision-making capacity.
- **Caregiver Support:**
 - Referred caregiver to local Alzheimer's Association support groups and respite care resources.
- **Follow-Up:**
 - Return to clinic in 3 months to evaluate medication tolerance and cognitive trajectory.

Electronically Signed:

Dr. Sarah Jenkins

Dr. Sarah Jenkins, MD
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St. Jude Medical Center — Division of Neurology